

NEXARA

The Intelligent Healthcare Continuity Ecosystem

Connecting every link in the care chain – from first symptom to long-term adherence.

SERIES A PITCH · 2026

CONFIDENTIAL

Nexara · Series A Pitch · 2026

Current Date: June 2026

At a Glance

Metric	Figure
Addressable market (US)	\$420B+ (care navigation + adherence + telehealth combined)
Preventable deaths annually	125,000+ (non-adherence alone)
Avoidable hospital costs	\$300B/year
Patients who struggle to reach the right specialist	~60% report seeing the wrong provider first
Chronic patients non-adherent	50%
Competitors with full-stack ecosystem approach	0

01 – The Problem

The healthcare chain is broken at every link.

Today's healthcare system forces patients to be their own navigators in a system too complex for even professionals to fully understand. The problems are not isolated – they compound each other across every stage of the care journey.

STAGE 1

Getting a Diagnosis

A patient feels unwell. They search symptoms online, get flooded with anxiety-inducing results, and either do nothing, go to an urgent care clinic that's not equipped for their problem, or see a GP who may not be the right specialist. The wrong first appointment delays real care by an average of **3-6 weeks** in common conditions, and **12+ months** in complex ones (rare diseases, autoimmune conditions, psychiatric disorders).

STAGE 2**Reaching the Right Professional**

Even when a patient gets a referral, the referral process is broken. Phone calls, fax machines, insurance verification, and 6-week wait times are the norm. **34% of specialist referrals are never completed** – patients get lost between the GP and the specialist. There is no intelligent triage, no routing logic, and no follow-up.

STAGE 3**The Care Chain Between Providers**

When multiple professionals are involved – a GP, a cardiologist, a pharmacist, a mental health professional – they rarely communicate in real time. A prescriber may not know what the diagnosing doctor said. A pharmacist may not know about a drug interaction introduced by a second specialist. The patient is the messenger, and messages get lost.

STAGE 4**Medication Adherence**

After diagnosis and prescription, 50% of chronic patients stop taking their medication within 6 months. The reasons are varied – cost, side effects, complexity, forgetfulness, disbelief – but the response from the healthcare system is uniform: silence. Nobody follows up. Nobody routes the problem to the right person.

The result: \$300B in avoidable costs, 125,000 preventable deaths, and millions of patients cycling through expensive interventions that could have been prevented with smarter routing and better continuity.

The Four Root Cause Clusters

Navigation failures

Patients can't identify the right type of care. AI-assisted triage is rare and fragmented. Symptom checkers exist, but none connect the assessment directly to a bookable, credentialed professional.

Referral drop-off

The referral pipeline has no intelligence. Referrals are sent to availability, not fit. There is no feedback loop telling the referring provider whether the patient ever arrived.

Care chain fragmentation

Providers don't share real-time context. Each appointment starts from scratch. The longitudinal story of a patient's health exists only in their memory.

Adherence gaps

Once a prescription is issued, the system assumes the patient is fine until they return with a complication. Nobody diagnoses **why** a patient stopped.

02 – Why Now

Five forces converge in 2026.

- **FHIR R4 CMS Mandate (Jan 2024)** – For the first time, EHRs must expose patient data through open APIs. Pharmacy data, clinical notes, and referral histories can be joined without bespoke integrations. The infrastructure for a full-stack ecosystem now exists at a standards level.
- **CMS Star Ratings & Financial Pressure** – Over \$3B in Medicare Advantage bonus payments are now tied directly to medication adherence PDC scores. Payers have an immediate financial incentive to invest in platforms that demonstrably improve outcomes.
- **AI Triage Maturity** – Large language models and causal ML are now accurate enough to perform meaningful preliminary routing – not diagnosis, but intelligent triage that improves first-contact accuracy and reduces inappropriate specialist referrals. The “wrong door” problem can now be partially solved algorithmically.
- **Post-COVID Telehealth Normalisation** – Video consultations have reached mainstream acceptance. Patients expect to be able to connect to a healthcare professional quickly and digitally. The infrastructure and cultural shift is complete; the smart layer on top has not been built.
- **IRA 2022 Drug Pricing Reforms** – Patient out-of-pocket cost sensitivity is higher than ever. Affordability is now a primary non-adherence driver, and platforms that can automatically surface copay assistance, generics, and patient support programs have a clear value proposition.

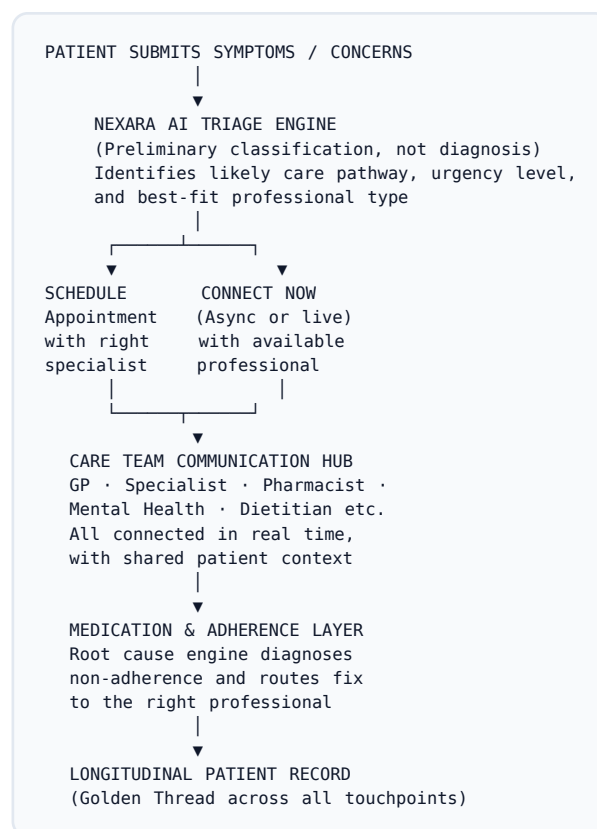
03 – The Solution

Nexara: the first intelligent healthcare continuity ecosystem.

Nexara is a B2B2C platform that connects the entire care chain – from a patient's first symptom to long-term medication adherence – using AI triage, smart provider routing, and a closed-loop communication layer across every professional in the care team.

It is not a symptom checker. It is not a booking platform. It is not an adherence app. It is the connective tissue that makes all of those things work together.

How the Platform Works



Core Platform Modules

STAGE 1

Nexara Triage Engine (AI-Assisted Navigation)

The patient enters symptoms, duration, prior history, and any relevant context in natural language or structured form. The Nexara AI engine performs a **preliminary classification** – not a diagnosis – across several dimensions:

- Likely care pathway (primary care, specialist, urgent care, mental health, pharmacy)
- Urgency tier (routine, priority, same-day, emergency redirect)
- Best-fit professional type (e.g. cardiologist vs. general internist vs. nurse practitioner)
- Risk flags (red-flag symptoms that require immediate escalation to emergency services)

The engine does not replace a doctor. It routes the patient to the right door before the consultation, dramatically improving first-contact accuracy.

Data inputs: patient-reported symptoms, free-text description, structured intake form, prior visit history (via FHIR), pharmacy fill history, wearable data (optional integration).

STAGE 2

Smart Provider Matching & Booking

Once triage classification is complete, Nexara surfaces the right professional from its verified provider network – matched on specialty fit, availability, location/telehealth preference, insurance acceptance, and patient-reported language or gender preference.

Patients can book a scheduled appointment directly, join an async consultation queue, connect immediately via video if a professional is available, or be routed to a pharmacist for medication questions without a full appointment.

Providers see why they are being matched – the triage context, prior notes, and patient-submitted intake – before the consultation begins. No more starting from scratch.

STAGE 3**Care Team Communication Hub**

For patients with complex or chronic conditions, Nexara creates a **shared care workspace** – a real-time channel where all professionals involved in a patient’s care can see updates, share notes, and coordinate without relying on the patient as the messenger.

This is not a generic messaging tool. It is structured around clinical context:

- A GP refers a patient to a cardiologist; the cardiologist’s notes sync back automatically
- A pharmacist flags a potential drug interaction; the prescriber gets an in-workflow alert
- A mental health professional updates their care note; the patient’s GP sees a summary flag
- A patient reports a side effect via the app; the prescriber gets a structured alert with one-click response options

All communication is HIPAA-compliant, EHR-embedded (SMART on FHIR), and auditable.

STAGE 4**Medication Adherence & Root Cause Engine**

Once a prescription is issued, Nexara's adherence layer activates. This is the most mature component of the platform, inherited and significantly expanded from the core Adherix research:

The **Root Cause Engine** uses pharmacy fill data, patient-reported check-ins, and side-effect sentiment analysis to classify **why** a patient is not adhering – across four barrier types:

- **Cost barrier** → auto-applies manufacturer copay coupons, surfaces generic alternatives, notifies prescriber
- **Side effect / experience barrier** → routes to pharmacist video call within 24h, structured alert to prescriber
- **Complexity barrier** → generates simplified dosing schedule, requests blister pack, sends structured reminder
- **Belief / fear barrier** → flags for targeted patient education, routes to care navigator

Each intervention is routed to the **right professional** automatically. Cost problems go to pharmacy support. Side-effect concerns go to the pharmacist or prescriber. Complexity issues trigger schedule simplification. Nothing falls through the cracks.

STAGE 5**Population & Practice Dashboard**

For health systems, payers, and practices:

- Real-time population adherence rates by drug, cohort, and risk tier
- PDC scores and CMS Star Rating impact projections
- Referral completion tracking (how many of your outbound referrals were completed?)
- Care chain gap analysis (where are patients falling out of the system?)
- RPM billing automation – auto-generates CMS-compliant documentation for Remote Patient Monitoring CPT codes 99453-99458

STAGE 6**Longitudinal Patient Health Record (Golden Thread)**

Every touchpoint – symptom submission, triage result, consultation, prescription, fill history, side-effect report, referral outcome – is woven into a single longitudinal narrative for each patient.

This is not a static health record. It is a living, structured story of the patient's health journey that is visible to every professional in their care team (with appropriate consent controls) and surfaced intelligently at each new touchpoint.

A cardiologist seeing a patient for the first time sees their full medication history, prior triage notes, and adherence patterns. A pharmacist fielding a side-effect call sees the patient's complete prescription context. The patient never has to repeat their history.

04 – Competitive Landscape

Nobody else owns the full stack.

The market is fragmented across several categories, each solving one part of the problem:

- **Symptom Checkers / Triage Apps** (Ada Health, Buoy Health, K Health) – these perform symptom assessment but do not connect to bookable professionals, do not integrate with care teams, and have no adherence layer.
- **Telehealth Platforms** (Teladoc, Hims & Hers, Babylon Health) – these connect patients to professionals but have no AI triage routing, no care team communication layer, no adherence tracking, and no longitudinal record.
- **Medication Adherence Point Solutions** (Medisafe, Wellth, AllazoHealth, TailorMed) – each solves one dimension of non-adherence. None integrate with the broader care chain.
- **Care Coordination Platforms** (Accolade, Quantum Health, Transcarent) – these are human-first care navigation companies using case managers and health advocates. High-touch, high-cost, not scalable.
- **EHR Referral Management** (ReferralMD, Relatient) – these are referral workflow tools within health systems. They improve referral completion rates but have no patient-facing triage layer or adherence tracking.

Full Competitive Matrix

Capability	Ada / Buoy	Teladoc	Medisafe	Allazo	TailorMed	Accolade	Nexara
AI symptom triage	✓	✗	✗	✗	✗	~	✓
Smart provider routing	✗	✗	✗	✗	✗	✓	✓
Live/async booking	✗	✓	✗	✗	✗	~	✓
Care team hub	✗	✗	✗	✗	✗	~	✓
Referral closed-loop	✗	✗	✗	✗	✗	~	✓
Root cause adherence AI	✗	✗	✗	✓	✗	✗	✓
Cost auto-resolution	✗	✗	✗	✗	✓	✗	✓
Side-effect detection	✗	✗	✗	~	✗	✗	✓
Prescriber alerts	✗	✗	✗	✗	~	✗	✓
FHIR-native EHR integration	✗	~	~	✗	✓	✗	✓
Pharmacy fill integration	✗	✗	~	✓	✓	✗	✓
Longitudinal narrative	✗	✗	✗	✗	✗	~	✓
Population dashboard	✗	✗	✗	✓	~	✓	✓
RPM billing automation	✗	✗	✗	✗	✗	✗	✓
Patient mobile app	✓	✓	✓	✗	✗	✗	✓

✓ = Full capability · ~ = Partial · ✗ = Not present

No competitor checks more than 5 of these 15 boxes. Nexara checks all 15.

05 – Business Model

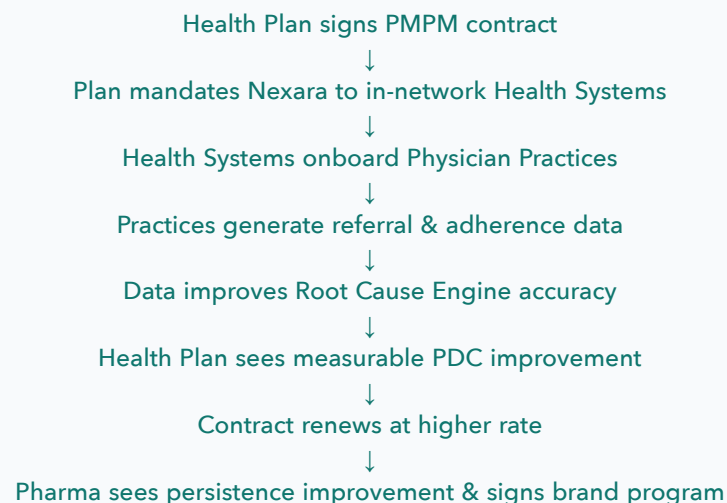
Five buyers. Outcome-aligned contracts across the full chain.

Nexara does not sell to patients. It sells to the stakeholders who pay the most when the care chain fails – and every contract is structured around measurable outcomes.

Customer	Core Pain	Value Delivered	Revenue Model	Unit Economics
Health Plans / PBMs	Non-adherence tanks Star Ratings; readmissions	PDC improvement, fewer readmissions, 4+ Star ratings	PMPM + shared savings	\$4-10 PMPM
Health Systems / ACOs	Referral drop-off, poor care, readmission penalties	Referral completion, care team comms, lower readmissions	Platform license + per-referral	\$15K-120K/yr per system
Pharma Manufacturers	Patients abandon specialty drugs; no data	More fills, therapy persistence, co-branded support	Per-brand SaaS + outcomes	\$250K-2.5M per brand
Pharmacy Chains	25-30% scripts unfilled; 15% abandoned mid-way	Fill rate uplift, reduced abandonment, loyalty	Platform license + fill bonus	\$0.60-1.50 per intervention
Physician Practices	No post-visit visibility; referral outcomes unknown	Closed-loop alerts, care coord, RPM billing	Per-provider SaaS	\$100-180 / provider / mo

Revenue Flywheel

The five buyer segments are not independent – they reinforce each other:



06 – Build Roadmap

30 months to full ecosystem.

Phase 1 --- Months 1--5: Core & MVP

Adherence + Triage foundations

- Pharmacy data ingestion (SureScripts/ NCPDP)
- Copay coupon auto-apply engine
- Patient symptom intake engine (NLP)
- Preliminary triage classification model
- Basic provider directory & booking
- FHIR R4 data ingestion pipeline
- HIPAA-compliant secure messaging

Target milestones: 2 pharmacy pilots, 1 telehealth network, 1 health plan POC.

Phase 2 --- Months 6--12: AI Core & Care Team

Intelligence and communication

- Root Cause Adherence Engine (4 barriers)
- SMART on FHIR apps (Epic, Oracle)
- Care team communication hub
- Referral closed-loop tracking
- Pharmacist video-call routing
- Ambient patient check-in & NLP sentiment
- Prescriber structured alerts
- Golden Thread longitudinal record v1

Target milestones: 3 health system pilots, FHIR certified for Epic App Orchard, 1st pharma brand contract.

Phase 3 --- Months 13--20: Payer GTM

Scale and outcomes evidence

- Population adherence dashboard
- Cohort-level care gap analysis
- First 2 shared-savings payer contracts
- AI Compliance Checker (ACC/ADA)
- RPM billing automation live (99453-8)
- Real-world evidence package for pharma
- Specialist-matching algorithm v2

Target milestones: 50,000+ active patients, proven PDC improvement data, Series A fundraise anchored on evidence.

Phase 4 --- Months 21--30: National Scale

Full ecosystem

- National pharmacy chains (CVS-scale)
- 10+ pharma brand programs
- 5+ regional payer shared-savings
- ACO & value-based care partnerships
- Wearable & remote monitoring (CGM etc.)
- AI Triage v3 (500K+ patient pathways)
- International expansion readiness

Target milestones: National footprint, fully integrated ecosystem with scale lock-in.

07 – Regulatory & Compliance Architecture

Built for scrutiny from day one.

Regulation	Nexara's Approach
HIPAA	End-to-end encryption, BAAs with all partners, audit-logged data access, zero data retention on third-party AI calls
FDA SaMD Guidance	Triage engine classified as decision support (not diagnosis), with mandatory human-in-the-loop escalation for any urgent flag
CMS / ONC FHIR Mandate	FHIR R4 native; SMART on FHIR app certification in progress for Epic App Orchard and Oracle Health Marketplace
21st Century Cures Act	Information blocking compliance by design; patient data portability built into the platform core
State Telehealth Laws	Provider network credentialing architecture supports per-state licensure verification and telehealth parity compliance

08 – Defensibility

Four compounding moats.

Data Network Effect (3-Way Flywheel)

Every patient interaction generates three data streams simultaneously: symptom-to-pathway accuracy data (improving triage), referral-to-outcome data (improving provider matching), and fill-to-adherence data (improving the Root Cause Engine). Each stream makes the other two more valuable. Competitors who build only one integration can never replicate the compounding accuracy of a 3-way flywheel.

EHR Embeddedness (Workflow Lock-in)

SMART on FHIR applications embedded in Epic and Oracle Health create deep workflow lock-in. Health systems build clinical processes around Nexara alerts and communication flows. Ripping it out requires an IT project, process retraining, and a budget cycle – typically a 2-3 year decision cycle even when a buyer wants to switch.

Outcomes-Based Contracts (Trust Moat)

Shared-savings structures mean Nexara is paid when health outcomes improve, not when software is installed. This creates auditable ROI evidence from every contract, which compounds into larger contracts and accelerates new sales. A pure-SaaS competitor cannot easily replicate this trust signal.

Causal AI Model (Data Advantage)

Diagnosing **why** a patient stopped their medication – not just **that** they did – requires a causal model trained simultaneously across all four barrier types, linked to clinical outcomes. This is a non-trivial ML problem requiring years of labeled data across pharmacy, clinical, and behavioral dimensions. A new entrant cannot shortcut this; they must collect the data first. Nexara will have a multi-year head start on the training set.

09 – Team Requirements

What we're building toward.

Clinical Credibility

At least one licensed clinician (MD, PharmD, or equivalent) on the founding team or advisory board, to navigate regulatory strategy, clinical partnership development, and provider trust-building.

Health Tech Infrastructure

Engineering leadership with prior experience in FHIR/HL7 integration, EHR app certification (Epic, Oracle Health), and HIPAA-compliant data architecture. This is a specialized skill set; general SaaS engineers cannot substitute.

AI / ML in Clinical Settings

A machine learning lead experienced in causal inference, NLP for clinical text, and the specific challenges of training models on sparse, noisy healthcare data (missing values, documentation inconsistency, label ambiguity).

GTM for Healthcare B2B

A commercial lead who has navigated health plan, health system, or pharma enterprise sales – ideally with existing relationships in at least one of the five buyer segments.

10 – The Ask

Series A: \$18M

Allocation	%	Use
Engineering & AI/ML	40%	Core platform build, Root Cause Engine training, FHIR integrations
Clinical & Regulatory	15%	FDA SaMD strategy, Epic App Orchard certification, clinical advisory
Sales & Partnerships	25%	Health plan, health system, and pharmacy chain BD; pharma brand pipeline
Operations & Compliance	10%	HIPAA infrastructure, legal, finance
Reserve	10%	18-month runway buffer

Target Outcomes by Series A Close (Month 20)

- **50,000+** active patients on platform
- **3+** health system pilot agreements (with measurable referral completion improvement)
- **2** payer shared-savings contracts with demonstrated PDC improvement data
- **1+** pharma brand program generating recurring revenue
- FHIR integration certified for at least one major EHR
- **\$2.5M+** ARR

Closing Thesis

The healthcare system has a connectivity problem, not just an information problem. The data exists. The professionals exist. The willingness to pay exists. What doesn't exist is the intelligent layer that connects every participant in the care chain – from the moment a patient notices a symptom to the moment they take their last pill in a complete course of treatment.

Nexara is that layer. Not a point solution. Not a single-stakeholder tool. An ecosystem – with AI triage that routes patients correctly, care team communication that keeps providers aligned, adherence intelligence that closes the last-mile gap, and a longitudinal record that makes every professional smarter on every subsequent encounter.

The \$420B problem is not one problem. It is four connected problems that have never been solved together. **Nexara solves them together.**